

SPARTAN COACHING

DISCIPLINE | EMPATHY | STRATEGY

MEDICARE HOSPICE REGULATIONS

Compliance Reference Guide for Hospice Sales Professionals

A plain-language summary of the Medicare Hospice Benefit, eligibility standards, disease-specific criteria, and compliance requirements relevant to sales conversations.

Medicare Hospice Regulations

What Every Hospice Sales Rep Needs to Know

THE MEDICARE HOSPICE BENEFIT: OVERVIEW

The Medicare Hospice Benefit was established in 1983 and is governed by 42 CFR Part 418. It is a comprehensive benefit that covers all services related to the patient's terminal diagnosis, including physician services, nursing visits, aide services, social work, counseling, spiritual care, medications, medical equipment, and bereavement support for families.

Unlike most Medicare benefits, hospice is an elected benefit. The patient or their legal representative must sign an election statement choosing hospice care and agreeing to forgo curative treatment for the terminal diagnosis. Patients can revoke and re-elect hospice at any time, and can receive unlimited periods of care as long as they continue to meet eligibility criteria.

THE FOUR CORE ELIGIBILITY REQUIREMENTS

Under 42 CFR 418.24, patients must meet all four of the following criteria to elect the Medicare Hospice Benefit:

1 Medicare Part A Enrollment

Patient must be enrolled in Medicare Part A (Hospital Insurance). This is typically automatic for people 65 and older who have worked at least 10 years in Medicare-covered employment. Patients under 65 may qualify through SSDI.

2 Terminal Illness Certification

A physician who is an employee or under contract with the hospice and the patient's attending physician (if the patient has one) must certify in writing that the patient has a terminal illness with a life expectancy of 6 months or less if the disease runs its normal course.

3 Patient Election of Hospice

The patient or their authorized representative must sign an election statement indicating they understand that choosing hospice means foregoing curative treatment for the terminal diagnosis. The statement must include the hospice name, acknowledgment of the palliative nature of the benefit, and acknowledgment of what services are not covered under the benefit.

4 Compliance with Care Plan

The patient must agree to follow the hospice plan of care and to receive care primarily through the hospice rather than from other Medicare providers for conditions related to the terminal diagnosis.

THE SIX-MONTH PROGNOSIS STANDARD

The six-month prognosis standard is widely misunderstood in the field, both by referral sources and by sales reps. Here is what you need to know and be able to explain clearly:

- Six months is a probability, not a deadline. The physician is certifying that IF the disease runs its normal course, death would be expected within six months. This is not a guarantee or a cutoff.
- Patients do not lose their hospice benefit if they live longer than six months. They can continue as long as they continue to meet eligibility criteria and a physician recertifies them at each benefit period.
- The benefit periods are: two initial 90-day periods, followed by unlimited 60-day periods thereafter. At each recertification, the clinical team must document continued decline or medical need.
- If a patient's condition improves to the point they no longer have a terminal prognosis, they are discharged from hospice. They can re-elect the benefit later if they decline again.

When referral sources say "the patient isn't sick enough for hospice," they are often applying a more restrictive standard than the law actually requires. Your job is to educate them on what the eligibility criteria actually say.

DISEASE-SPECIFIC ELIGIBILITY CRITERIA

Congestive Heart Failure

- NYHA Class IV: symptoms of heart failure at rest or with any physical activity
- Ejection fraction at or below 20% despite optimal medical treatment
- Three or more hospitalizations for CHF exacerbation in the prior 12 months
- Persistent resting hypotension (systolic below 90) despite treatment
- Cachexia, renal insufficiency, or hyponatremia indicating multi-organ involvement
- Patient declining or not tolerating evidence-based therapies such as ACE inhibitors or beta blockers

COPD and Pulmonary Disease

- Disabling dyspnea at rest or with minimal exertion despite maximal bronchodilator therapy
- FEV1 below 30% of predicted value on pulmonary function testing
- Hypoxemia at rest requiring supplemental oxygen at 88% or less on room air

- Hypercapnia with PCO₂ 50 mmHg or greater on arterial blood gas
- Right heart failure secondary to pulmonary disease (cor pulmonale)
- Progressive weight loss or cachexia exceeding 10% body weight over 6 months

Cancer

- Metastatic disease or locally advanced disease not responsive to further treatment
- Patient has declined further curative treatment or is no longer a candidate
- Palliative Performance Scale (PPS) at or below 70% indicating meaningful functional limitation
- Rapid clinical decline over 4 to 6 weeks with weight loss, weakness, or altered cognition
- Symptomatic or laboratory evidence of active disease despite treatment

Dementia and Alzheimer Disease

- FAST Scale Stage 7 or beyond: severe cognitive decline with marked functional limitation
- Unable to ambulate, dress, bathe, or toilet without complete assistance
- Urinary and fecal incontinence consistently
- Limited meaningful verbal communication: fewer than 6 intelligible words per day
- Plus at least one complication in the prior 12 months: aspiration pneumonia, pyelonephritis, septicemia, decubiti Stage 3 or 4, or recurrent fever despite antibiotics

Renal Disease

- Patient not seeking dialysis or electing to discontinue dialysis
- Creatinine clearance below 10 ml per min (15 for diabetics)
- Serum creatinine above 8.0 mg per dl (6.0 for diabetics)
- Signs of uremia: confusion, restlessness, intractable nausea, reduced urine output

Neurological Disease (ALS, Stroke, Parkinson's)

- For ALS: critically impaired breathing with vital capacity below 30% predicted, dysphagia with weight loss, and at least one complication in the prior year
- For CVA: coma or persistent vegetative state beyond 3 days, dysphagia, or post-stroke dementia meeting FAST Stage 7 criteria
- For Parkinson's: Stage 3 or beyond with significant functional decline and complications

COMPLIANCE LANDMINES IN SALES CONVERSATIONS

Certain conversations and actions can create legal and regulatory risk. Every Spartan-trained rep understands where the compliance lines are and stays well clear of them.

- Never offer gifts, meals, entertainment, or anything of value to referral sources or their staff that exceeds the Anti-Kickback Statute safe harbor thresholds. As of 2026 the per-item limit is \$21 and the annual aggregate per person limit is \$420.
- Never imply that a patient qualifies for hospice without a clinical evaluation. You can educate on criteria but the eligibility determination belongs to the clinical team.
- Never pressure a facility to refer patients who have not been clinically evaluated, or suggest targets for the number of referrals they should be sending.
- Never discuss patient-specific clinical information with referral sources unless you have appropriate authorization. PHI protections under HIPAA apply to sales conversations.
- Never make guarantees about outcomes, length of stay, or the patient experience. These are clinical matters outside the scope of a sales conversation.

COMPLIANCE IS COMPETITIVE ADVANTAGE

The best hospice sales reps do not view compliance as a constraint. They view it as a differentiator. When you educate referral sources accurately, avoid questionable practices, and demonstrate clinical credibility, you build trust that competitors who cut corners will never achieve. Compliance is how the Spartan Method builds lasting market position.